

VOKER[®] FILM COATED TABLET

Famotidine, the active ingredient of Voker F. C. Tablet, is a histamine H₂-receptor antagonist. Chemically, famotidine is 3-[2-(diaminomethylethylamino) thiazol-4-ylmethylthio]-N-sulphamoyl-propionamide .

Ingredient(s):

Voker F. C. Tablet 20mg

Each tablet contains:

Famotidine 20mg

Voker F. C. Tablet 40mg

Each tablet contains:

Famotidine 40mg

Pharmacology (Summary of Pharmacodynamic and Pharmacokinetics):

Pharmacodynamic:

1. Famotidine is a potent inhibitor of gastric acid secretion acting at the H₂-receptor site. It is a competitive antagonist with high selectivity and binding affinity.
2. Basal acid output is diminished, as is maximal acid output stimulated by pentagastrin or food. In addition to the reduction in acid output, the volume of gastric secretion is decreased in basal, nocturnal and stimulated states.
3. Famotidine is 40 times as potent as cimetidine with a duration of action 1~ 3 times as long. Famotidine has 7 ~ 10 times the potency of ranitidine. 40mg of famotidine is roughly equivalent to 300mg of ranitidine in reducing basal and nocturnal acid secretion by at least 75% for up to 12 hours. The duration of acid suppression with famotidine is longer than that with ranitidine.
4. Famotidine has no anti-androgenic activity and has no effects on hepatic cytochrome P450 activity.

Pharmacokinetics:

1. In normal subjects, 20mg of famotidine suppressed pentagastrin-stimulated acid output by 90%, two hours after oral administration; at 12 hours, output was still only 50% of control. A 10mg dose suppressed acid output by 70%, and 5mg by 60% at 2 hours. Famotidine is considerably more potent than cimetidine, the inhibitory effect of 5mg famotidine were comparable to 300mg of cimetidine.
2. After oral administration, absorption is rapid and the onset of action is by one hour, approximately 40% of an oral dose is absorbed. Peak plasma concentrations occur about 2 hours after dosing.
3. Famotidine has minimal pre-systemic metabolism. 15 ~ 20% of the drug is bound to plasma proteins. The plasma half-life of the drug is approximately 3 hours, but the duration of action is longer.
4. Famotidine is excreted in the urine largely unchanged, and this represents that major mode of elimination of the drug.

Indication(s):

For the treatment of benign gastric and duodenal ulcers, reflux oesophagitis and Zollinger-Ellison Syndrome.

Dosage and Administration:

Duodenal and benign gastric ulcer: Initial daily dosage is 40mg at bedtime for up to 4~8 weeks, maintenance: 20mg at night time.

Zollinger-Ellison Syndrome initially 20mg every 6 hours; maximum of 800mg daily. Reflux oesophagitis: 20 to 40mg twice daily for 6 to 12 weeks.

Dosage adjustment is required for patients with moderate to severe renal insufficiency. Since CNS adverse effects have been reported in patients with moderate to severe renal insufficiency, to avoid excess accumulation of the drug, the dose of famotidine may be reduced to half the recommended dose or the dosing interval may be prolonged to 36-48 hours as indicated by the patient's clinical response.

Route of administration: Oral

Contraindication(s):

Hypersensitivity to any component of this product.

Precaution(s) / Warning(s):

1. Symptomatic response to therapy with Famotidine does not preclude the presence of gastric malignancy.
2. For patients with severe renal insufficiency (creatinine clearance rates below 30ml/min), dosage reduction is recommended to compensate for the slower elimination of the drug.
3. There are no adequate or well-controlled studies in pregnant women, this drug should be used during pregnancy only if clearly needed.
4. Studies performed in lactating rats have shown that Famotidine is secreted into breast milk. A decision should be made whether to discontinue nursing or discontinue the drug, taking into account the importance of the drug to the mother.
5. Pediatric Use: Safety and effectiveness in children have not been established.
6. As elderly patients are more likely to have decreased clearance of famotidine. Care should be taken in dose selection and it may be useful to monitor renal function.

Drug Interaction(s):

No drug interactions of clinical importance have been identified. Voker does not interact with the cytochrome P450-linked drug metabolizing enzyme system. Compounds metabolized by this system which have been tested in man included warfarin, theophylline, phenytoin, diazepam, propranolol, aminopyrine and antipyrine. Indocyanine green as an index of hepatic blood flow and/or hepatic drug extraction has been tested and no significant effects have been found.

Studies in patients stabilized on phenprocoumon therapy have shown no pharmacokinetic interaction with famotidine and no effect on the pharmacokinetic on anticoagulant activity of phenprocoumon.

Pregnancy and Lactation:

There are no adequate or well-controlled studies in pregnant women, this drug should be used during pregnancy only if clearly needed.

Studies performed in lactating rats have shown that Famotidine is secreted into breast milk. A decision should be made whether to discontinue nursing or discontinue the drug, taking into account the importance of the drug to the mother.

Side Effect(s) / Adverse Reaction(s):

Gastrointestinal	: Constipation, diarrhea, vomiting, abdominal discomfort, anorexia, dry mouth, liver enzyme abnormalities.
Nervous System	: headache, dizziness, psychic disturbances.
Cardiovascular	: palpitations, arrhythmia, AV block
Respiratory	: bronchospasm
Body as a whole	: fever, asthenia, fatigue
Skin	: pruritus, dry skin, flushing, acne
Special senses	: tinnitus, taste disorders.

Symptoms and Treatment for Overdosage, and Antidote(s):

There is no experience to date with deliberate overdosage. Signs of acute intoxication in I.V. treated dogs were emesis, restlessness, tachycardia and collapse.

Doses of up to 640mg/day have been given to patients with pathological hypersecretory condition with no serious adverse effects. In the event of overdosage, treatment should be symptomatic and supportive. Unabsorbed material should be removed from the gastrointestinal tract, the patients should be monitored, and supportive therapy should be employed.

Shelf-Life:

3 years from the date of manufacture.

Storage Condition(s):

Store at temperature below 30°C. Protect from light and moisture.

Product Description and Packing(s):**Voker F. C. Tablet 20mg:**

An orange film coated square tablet. “”

Blister packing of 10's x 6, 10's x 10 and 10's x 50.

Voker F. C. Tablet 40mg:

An orange film coated round tablet, one side is impressed with a score. “”

Blister packing of 10's x 6, 10's x 10 and 10's x 50.



Manufacturer and Product Registration Holder:
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